Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 1.0 | **Date:** 03-Apr-2026

---

1. Administrative & Study Identification

Full Study Title: A Phase Ib, Open-Label, Multicenter Study Evaluating the Safety, Pharmacokinetics, and Efficacy of Mosunetuzumab or Glofitamab in Combination With CC-220 and/or CC-99282 in Patients With B-Cell Non-Hodgkin Lymphoma **Official Title:** A Study Evaluating the Safety, Pharmacokinetics, and Efficacy of Mosunetuzumab or Glofitamab in Combination With CC-220 and/or CC-99282 in Participants With B-Cell Non-Hodgkin Lymphoma **Short Title/Acronym:** CO43805 (B-Cell NHL Immunotherapy Combination) **Protocol Number:** CO43805 / 2023-505185-28-00 **NCT Number:** NCT05169515 **Posting ID:** 245589492834216613 **Trial Status:** Recruiting **Sponsor / Company Name:** Hoffmann-La Roche / Roche **Investigational Product:** Mosunetuzumab, Glofitamab, CC-220 (Iberdomide), CC-99282 (Golcadomide), and Obinutuzumab (pre-treatment) **Phase of Development:** PHASE1 (Phase Ib) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

---

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and to establish the Maximum Tolerated Dose (MTD) and/or Recommended Phase 2 Dose (RP2D) of mosunetuzumab or glofitamab in combination with CC-220 and/or CC-99282 in participants with relapsed/refractory B-Cell Non-Hodgkin Lymphoma (B-Cell NHL). **Secondary Objectives:** To evaluate preliminary efficacy (Overall Response Rate, Complete Response rate, Duration of Response), pharmacokinetics (PK), and pharmacodynamics of the combinations.

2.2 Background & Rationale Distinct from Hodgkin lymphoma both morphologically and biologically, non-Hodgkin lymphoma (NHL) is characterized by the absence of Reed-Sternberg cells, can occur at any age, and usually presents as a localized or generalized lymphadenopathy associated with fever and weight loss. The clinical course varies according to the morphologic type. NHL is clinically classified as indolent, aggressive, or having a variable clinical course. NHL can be of B-or T-/NK-cell lineage. This study addresses the high unmet medical need in relapsed/refractory (R/R) B-cell NHL. Bispecific antibodies targeting CD20 and CD3 (mosunetuzumab and glofitamab) effectively redirect T-cells to destroy malignant B-cells. Combining these therapies with novel cereblon modulators (CELMoDs: CC-220 and CC-99282) may enhance T-cell activation and proliferation, providing deeper, longer-lasting responses.

---

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose Escalation and Dose Expansion cohorts) **Blinding:** Open-label **Randomization:** N/A (Assigned to specific combination cohorts)

3.2 Planned Enrollment & Duration Target N: 121 participants **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 26-Oct-2023 **Estimated Primary Completion:** Not specified (Total participant duration is roughly 3 years)

---

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with an Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2. 2. Histologically documented hematologic malignancies expected to express the CD20 antigen. Availability of a representative tumor specimen and the corresponding pathology report for confirmation of the diagnosis of NHL (a fresh pretreatment biopsy during the screening period is preferred). 3. Fluorodeoxyglucose-avid lymphoma (i.e., PET-positive lymphoma). 4. At least one bi-dimensionally measurable nodal lesion (> 1.5 cm) or extranodal lesion (> 1.0 cm) by diagnostic quality CT or PET/CT scan. 5. Must have relapsed after or failed to respond to prior systemic therapy based on the following: * **Dose Escalation:** At least two prior lines of systemic therapy. * **Dose Expansion (FL Grades 1-3a):** At least one prior line of systemic therapy and must require systemic therapy. * **Dose Expansion (DLBCL/transformed FL):** At least one prior systemic treatment regimen. If only one prior line received, participants must not be considered a candidate for autologous stem cell transplantation (ASCT) or must have refused ASCT, and must be ineligible for or unable to receive chimeric antigen receptor T-cell (CAR-T) therapy. 6. Agreement to strict pregnancy prevention protocols for both male and female participants due to teratogenic risks.

4.2 Exclusion Criteria 1. History of autoimmune disease (e.g., myocarditis, pneumonitis, rheumatoid arthritis, multiple sclerosis). 2. Major surgery or significant traumatic injury < 28 days prior to enrollment, or anticipation of the need for major surgery during study treatment. 3. Current or past history of central nervous system (CNS) lymphoma or leptomeningeal infiltration. 4. QTc interval of > 470 ms. 5. Inability to swallow pills, or persistent diarrhea or malabsorption $\ge$ Grade 2, despite medical management. 6. Prior therapy with cereblon (CRBN)-modulating drug (e.g., lenalidomide, avadomide/ CC-122, pomalidomide) $\le$ 4 weeks prior to starting CC-220 and/ or CC-99282. 7. For the glofitamab cohort: Documented refractoriness to an obinutuzumab monotherapy-containing regimen (did not achieve PR/CR or progressed within 6 months of last dose).

---

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Group A:** Mosunetuzumab on Days 1, 8, 15 of Cycle 1, then Day 1 of Cycles 2–12 + CC-220 or CC-99282 capsules taken once daily. * **Group B:** Glofitamab on Days 8 and 15 of Cycle 1, then Day 1 of Cycles 2–12 + CC-99282 capsules once daily. (Includes a one-time pre-treatment dose of Obinutuzumab on Day 1 of Cycle 1 to minimize toxicity). **Route of Administration:** Subcutaneous (Mosunetuzumab), Intravenous (Glofitamab, Obinutuzumab), Oral (CC-220, CC-99282). **Duration of Treatment:** Up to 12 cycles. Cycle 1 is 21 days; Cycles 2–12 are 28 days each.

5.2 Concomitant & Prohibited Therapy Permitted: Tocilizumab (Trade Name: Actemra, ATC Code: L04AC07) for the management of Cytokine Release Syndrome (CRS). Standard medications for co-morbidities. **Prohibited:** Concurrent investigational therapies; live, attenuated virus vaccines during the study and for 5 months after final dose; immunosuppressive agents not directed by protocol.

---

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, PET/CT Scan, Fresh Biopsy, Eligibility Review
Cycle 1	Day 1 to 21	Obinutuzumab pre-treatment (if Group B), First Doses, Intense PK/PD Labs, CRS Monitoring
Cycles 2-3	Day 1 to 28	Weekly to bi-weekly visits: Safety Labs, Dosing, Efficacy Assessment
Cycles 4-12	Day 1 to 28	Every 3-4 weeks: Maintenance Dosing, Safety checks, Tumor Assessments
End of Study	Follow-up	Final Vital Signs, Exit Interview, Survival & Disease Progression Tracking

---

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. Percentage of participants with dose-limiting toxicities (DLTs) [dose escalation]. 2. Percentage of participants with adverse events [all cohorts]. 3. Best overall response rate (ORR), defined as the proportion of participants whose best overall response is a partial response (PR) or a complete response (CR) during the study, as determined by the investigator using Lugano 2014 criteria [dose expansion].

7.2 Secondary & Exploratory Endpoints 1. Best CR rate, defined as the proportion of participants whose best overall response is a CR during the study, as determined by the investigator using Lugano 2014 criteria [all cohorts]. 2. Best ORR (CR or PR at any time) on study as determined by the investigator using Lugano 2014 criteria [dose escalation]. 3. Duration of response (DOR) as determined by the investigator using Lugano 2014 criteria [all cohorts]. 4. **Pharmacokinetics:** $Cmax$ and AUC of mosunetuzumab, glofitamab, CC-220, and CC-99282.

---

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular clinical visits. Specific, intense monitoring algorithms are in place for Cytokine Release Syndrome (CRS) and Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS). **Serious Adverse Events (SAEs):** Reported to the sponsor within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Internal safety monitoring committee analyzing DLTs and safety profiles before dose-escalation decisions.

---

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving ≥ 1 dose of any study drug); Efficacy Evaluable Set (patients with measurable disease and ≥ 1 post-baseline scan). **Sample Size Justification:** Target $N = 121$. Adaptive Phase 1b sample size calculated to adequately support dose escalation rules and provide early efficacy signaling in dose expansion. **Handling of Missing Data:** No imputation for missing safety data. Time-to-event efficacy endpoints (PFS, OS, DOR) will utilize standard right-censoring methods.

---

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring of safety data to assure protocol adherence, particularly focusing on DLT observation periods. **Audit Trail:** Validated Electronic Data Capture (EDC) system with strict audit trails, eCRF locking, and standard data clarification form (DCF) generation.

---

11. Study Identifiers & Governance

Primary ID: CO43805 **Registry Number:** NCT05169515 **Posting ID:** 245589492834216613 **Sponsor/Lead Organization:** Hoffmann-La Roche / Roche **Official Regulatory Title:** A Phase Ib, Open-Label, Multicenter Study Evaluating the Safety, Pharmacokinetics, and Efficacy of Mosunetuzumab or Glofitamab in Combination With CC-220 and/or CC-99282 in Patients With B-Cell Non-Hodgkin Lymphoma

---

12. Clinical Framework (Lymphoma Specific)

Primary Condition: Non-Hodgkin Lymphoma (Preferred Name: Lymphoma, Non-Hodgkin; UMLS: B-Cell Non-Hodgkin Lymphoma) **Disease Area:** Non-Hodgkin Lymphoma **Semantic Type:** Neoplastic Process **Terminology Codes:** * **MeSH Code:** D008228 * **HPO Code:** HP:0012539 * **SNOMED ID:** 1172592001 * **SNOMED Hierarchy:** 1163043007 | Malignant lymphoma | Malignant lymphoma (morphologic abnormality) **Diagnosis Threshold:** Relapsed after or failed to respond to prior systemic therapy lines as strictly defined by cohort (dose escalation vs. expansion). **Medical Methodology:** Bispecific T-cell engaging antibodies + Cereblon Modulators (CELMoDs). **Optimization Target:** Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D) and clinical remission (ORR/CR/DOR per Lugano 2014 criteria).

---

13. Study Procedures & Methodology

Management Pathway: Targeted immunotherapy and continuous step-up dosing paradigm for toxicity mitigation. **Education Protocol (HCP):** Mandatory site training on grading and immediate management of CRS and ICANS (including the administration of tocilizumab). **Education Protocol (Patient):** Counseling on mandatory pregnancy prevention (due to teratogenic potential of CELMoDs) and prompt symptom reporting for CRS. **Quality Audit Mechanism:** Continuous safety review by the medical monitor and regular central review of PET/CT efficacy scans.

---

14. Observation & Follow-Up Schedule

Total Duration: Up to 3 years total (up to 12 treatment cycles + survival follow-up) **Onsite Visit Cadence:** Weekly during first 2–3 cycles, then every 3-4 weeks (per cycle frequency) **Remote Monitoring Frequency:** Regular interval tracking for survival status after treatment discontinuation **Checklist Review Interval:** Safety and dosing compliance checks conducted every cycle

---

15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	**[DD-MMM-YYYY]**
Clinical Operations Lead	**[Name]**	___	***[DD-MMM-YYYY]***
Lead Statistician	***[Name]***	_____	[DD-MMM-YYYY]